

CONTRACEPTION

Give emergency contraception if patient had unprotected sex in past 5 days and does not want pregnancy:

- Give as soon as possible single dose **levonorgestrel** 1.5mg orally **or** if patient wanting long-term contraception, check pregnancy test and if negative, insert **Copper IUD**¹ instead.
 - If patient > 80kg, BMI² ≥ 30, or on efavirenz, rifampicin, phenytoin or carbamazepine, increase dose of **levonorgestrel** to 3mg.
 - Give **metoclopramide** 10mg 8 hourly as needed for nausea/vomiting. If patient vomits < 2 hours after taking levonorgestrel, repeat dose.
 - Offer to start long-term contraceptive at same visit (if IUD not chosen). If injectable or implant given, check pregnancy test in 2 weeks.
- Advise patient to return for pregnancy test if next period is more than 1 week late.
- Consider need for HIV and hepatitis B post-exposure prophylaxis ↪ 108.

Assess the patient starting and using contraception

Assess	When to assess	Note
Symptoms	Every visit	Check for symptoms of STIs: vaginal discharge, ulcers, lower abdominal pain. If present ↪ 49. Manage other symptoms as on symptom pages.
Chronic conditions	First visit	Check suitability of method if any of: cancer (especially breast, cervical, uterine), blood clots, hypertension, CVD, stroke, ischaemic heart disease, diabetes, liver disease, migraines, or unexplained vaginal bleeding ↪ 156.
Medication	Every visit	If on ART, TB or epilepsy treatment, check method is suitable ↪ 156.
Periods	Every visit	Ask about periods: interval between periods, blood loss, number of days period lasts, dysmenorrhoea (pain/cramps associated with periods).
Menopause	If > 40 years: yearly	Ask about menopausal symptoms: hot flushes, night sweats, vaginal dryness, mood changes, difficulty sleeping. If menopausal ↪ 169.
Sexual health	Every visit	If risky sexual behaviour: new or multiple partner/s, uses condoms unreliably, has sex under influence of alcohol/drugs, give safe sex advice.
Adherence	Every visit	• If already on contraceptive, ask about concerns and satisfaction with method. If unsatisfied, consider different contraceptive, rather than risking unwanted pregnancy. • If patient has missed injections or pills, manage ↪ 155.
Side effects	Every visit	If already on contraceptive, ask about and manage side effects ↪ 155.
Weight (BMI ²)	Every visit	If BMI > 25, assess CVD risk ↪ 127.
BP	First visit; every visit if on pill or injectable	• Check BP: if ≥ 140/90 ↪ 132. • If hypertension or BP ≥ 140/90, avoid/change from combined oral contraceptive. If BP ≥ 160/100, also avoid/change from injectable.
Breast check	First visit, then yearly ³	Check for lumps in breasts ↪ 43 and axillae ↪ 25.
Pregnancy	Every visit	• Before starting contraception, exclude pregnancy ↪ 157. • If pregnancy suspected (nausea/breast tenderness or missed period when using combined oral contraceptive), exclude pregnancy ↪ 157.
HIV	Every visit	Test for HIV ↪ 110. If positive, give routine HIV care ↪ 111. If negative, consider need for PrEP, if available ↪ 106.
Cervical screen	When needed	If HIV negative: do 3 cervical screens, each 10 years apart from age 30 ↪ 55; if HIV positive: do cervical screen at HIV diagnosis (regardless of age), then 3-yearly ↪ 55.

Advise the patient starting/on contraception

Health for All

↪ 51

- Educate patient to use contraceptive reliably. Advise to discuss concerns/problems with method and find an alternative, rather than stopping it and risking unwanted pregnancy.
- Recommend dual contraception: one method of contraception *plus* condoms to protect from STIs and HIV. Demonstrate and give male/female condoms.
- Discuss risky sexual behaviour. Explain risks of pregnancy and infections. Encourage patient to have 1 partner at a time and if HIV negative to test for HIV between partners. Advise that partner/s test too.
- Educate about the availability of emergency contraception (see above) and termination of pregnancy ↪ 158 to prevent unwanted pregnancy. Also educate about PrEP, if available.
- Advise patient on pill or implant to tell clinician if starting ART, TB or epilepsy treatment as these may interfere with pill or implant effectiveness.
- If on pill: if vomits within 2 hours, or severe diarrhoea within 12 hours of taking pill, repeat dose as soon as possible. If persistent vomiting/diarrhoea > 24 hours, advise to use condoms or abstain during illness and for 7 days after resolved.

¹The newly available **levonorgestrel IUD 52mg (LNG-IUD)** is not coded for emergency contraception as yet. ²BMI = weight (kg) ÷ height (m) ÷ height (m). ³If patient > 40 years old: check breasts 6 monthly.